
CONSULT FOR AFRICA

Dr Bola Akinola: Website Rebuild Brief and Draft Copy

Rebuilding bolarinwaakinola.com around the complex-case authority

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How to use this document

This is two things in one: a **brief** for whoever builds the site (designer and developer), and **draft copy** ready to drop in and edit. The copy is written to be his voice, not marketing gloss. Anything in [square brackets] is a placeholder for a fact or number to confirm, most of which the discovery questionnaire will supply. Every claim about numbers or outcomes must be true and his own before it goes live.

The site is the flagship of the **primary brand (Dr Bola Akinola)**, with **Osteon Clinics presented as the endorsed place of delivery**, never the other way round. The URL stays bolarinwaakinola.com.

1. What the site has to do

The current site leads with a generic "care you can trust, every step of the way," splits patients into Nigeria and international, mentions robotic knee, and buries the one thing that is actually the moat: the complex, revision and infection work. It has no visible outcomes, no proof, no testimonials, and no pathway for the referring doctors who are the real engine of this practice.

The rebuild has four jobs, in order:

1. **Claim the complex-case authority in the first screen.** A visitor should understand within five seconds that this is the surgeon for the hard cases, revision, infection, complex trauma, and the cases that went wrong elsewhere.
2. **Convert the two real buyers:** the patient (or their diaspora child paying), and the referring doctor. These need different pathways, and the site currently serves neither well.
3. **Prove it.** Outcomes, volumes, credentials, and real patient stories, shown not asserted. This is what closes the gap against a competitor who leads with "4,000+ procedures."
4. **Carry the honest-counsel voice throughout**, so the whole site feels like a surgeon who tells the truth, including when not to operate.

What we are deliberately not doing: competing head-on with "World-Class Orthopedics, Here at Home." That flag is taken. We lead with the complex-case authority and let world-class-care-at-home be the closing reassurance, attached to the hard cases.

2. Site architecture

A tight, purposeful set of pages. Depth where it earns trust, nothing decorative.

Page	Job	Primary audience
Home	Claim the position, route the two buyers, show proof fast	All
Complex & Revision Surgery	The hero clinical page: revision, infection, failed/foreign surgery, limb reconstruction	Patients + referrers
Hip & Knee Replacement	Primary elective, reassurance, what to expect	Patients
About Dr Bola	The credentials, the story, the ethics-forward voice	All
Patient Stories & Outcomes	Proof: testimonials, before/after, numbers	Patients



For Referring Doctors	The public face of the portal: the referral pathway, promise, MDT and fellowship, with a login	Doctors
Clinical Community Portal (gated)	Doctor-only login: refer and track, join MDTs, learn, become a fellow	Doctors
For Patients Abroad / Diaspora	The recapture and diaspora-funded pathway	Diaspora
Plan Your Care	The single conversion hub: how to book, what it costs, what happens next	Patients
Insights (light)	A few authority pieces for SEO and thought leadership	All + search
Contact	Osteon location, visiting sites, WhatsApp, secure enquiry	All

Global calls to action, present on every page: **"Book a consultation"** (patient) and a quieter **"Refer a patient"** / **"Doctor login"** (doctor).

The Clinical Community Portal is a major, separate workstream, not just a page. It is the doctor-only, login-gated platform where doctors refer and track patients, join complex-case MDTs, learn from Dr Bola, and progress to the OrthoSurplus fellowship. The public "For Referring Doctors" page is its shop window; the portal itself is the product. It is specified in full in the companion document, *The Clinical Community Portal*, and should be built in phases (referral MVP first). The website and the portal share one login and one design system.

3. Voice and design direction

Voice. Senior, calm, precise, humane. The register of a consultant who has seen the hard cases and does not need to boast. Short sentences. Plain English over jargon, with the medical term in brackets where it reassures. Honesty as a feature: he will tell you when surgery is not the answer. No hype, no superlatives stacked on superlatives, and no em dashes anywhere in the copy.

Design. Premium, clinical, uncluttered. White space, one strong accent, real photography of him and the work (not stock). Legible at speed. Trust signals visible without scrolling: FRCS, the fellowships, the institutions. Mobile-first, because most Nigerian traffic and most diaspora browsing is on a phone. Fast: this must load quickly on a mid-tier Android on Nigerian mobile data.

Proof furniture that should recur across pages: credential row (FRCS, BOA fellowship, MPH, years, countries), a live outcomes strip [numbers to confirm], and at least one real patient quote.

4. Draft copy

4.1 Home

Hero (above the fold)



Headline: The surgeon others send their hardest cases to.

Subhead: Hip and knee replacement, revision surgery, and complex reconstruction in Lagos.
UK-trained, twenty years, three countries. When a case is difficult, or a previous operation has failed, this is where it gets put right.

Primary button: Book a consultation **Secondary link:** Refer a patient

Credential strip (immediately below hero)

FRCS (Tr. & Orth.), UK • BOA Fellowship in Arthroplasty & Trauma • MPH, London • 20+ years across three countries

Section: What I do

I am an orthopaedic and reconstructive surgeon. Most of my work is the hip and knee: replacing worn joints, and, increasingly, fixing the cases that have gone wrong somewhere else. Failed replacements. Infected joints. Fractures that will not heal. Reconstruction after complex trauma. These are the operations many surgeons would rather not take on. They are the ones I trained for.

[Link] See complex and revision surgery >

Section: Three reasons patients and doctors choose this practice

Trained for the hard cases. A UK Fellowship (FRCS Tr. & Orth.) and a national leadership fellowship in arthroplasty and trauma, built specifically around revision and complex work.

Told the truth. Not everyone who is sent to me needs surgery. I will tell you plainly what will help and what will not, before anything is booked.

World-class, without leaving Nigeria. For the cases people used to fly abroad for, the expertise is now here, at Osteon Clinics in Lagos and at partner hospitals.

Section: Proof (outcomes strip)

[X] joint replacements and revisions • [X] complex reconstructions • [X years] of practice • patients seen at Osteon, Cedarcrest and Diamed

(All figures to be confirmed and kept accurate.)

Section: Patient story teaser

[One short, real patient story: the person, the problem, the outcome, in their words or his. Links to Patient Stories.]

Closing band



You do not have to travel for this. If you have been told you need a joint replacement, a revision, or that your case is too complex, let us look at it properly first.

Button: Plan your care

4.2 Complex & Revision Surgery (the hero clinical page)

Headline: Complex, revision and salvage surgery

Intro: Some cases are harder than others. A joint replacement that has worn out, loosened or become infected. A fracture that has not united. A limb that needs reconstruction after serious injury. An operation done elsewhere, sometimes abroad, that has not gone to plan. These cases need a surgeon who does them often, not occasionally. That is the core of my practice.

What I treat

- **Revision hip and knee replacement.** Redoing a joint replacement that has failed, loosened, worn or dislocated.
- **Bone and joint infection.** Diagnosing and clearing infection in and around joints and implants, one of the most demanding areas in orthopaedics.
- **Failed or complicated surgery from elsewhere.** Including cases operated on abroad that have returned with a problem. It is often possible to put these right.
- **Complex trauma and limb reconstruction.** Rebuilding after severe fractures, non-union and deformity.

Why this is specialist work

Revision and infection surgery is not a bigger version of a routine replacement. It is a different discipline: harder to plan, harder to execute, and far less forgiving. It rewards a surgeon who has done a great many of them and who works to an international standard. That is what I offer, and it is why other doctors refer their most difficult cases here.

A note on second opinions

If you have been told that nothing more can be done, or that your only option is to travel abroad, it is worth a second opinion before you decide. Many complex cases are more treatable than they first appear.

Button: Book a second opinion

4.3 Hip & Knee Replacement (primary elective)

Headline: Hip and knee replacement, done properly

Intro: For an arthritic hip or knee that no longer responds to non-surgical care, a joint replacement can give you back a life without constant pain. It is one of the most successful operations in modern medicine, when it is done well and for the right patient.

Is it time?



Surgery is not always the answer, and I will tell you honestly if it is not yet time for yours. Replacement is right when pain and stiffness limit your daily life and simpler measures have stopped working. We will go through your options together before deciding anything.

What to expect [short, plain, reassuring: assessment, the operation, robotic-assisted option where appropriate, recovery, aftercare]

Aftercare that does not end at discharge [the follow-through: rehabilitation, review, being reachable]

4.4 About Dr Bola

Headline: About Dr Bola Akinola

I am a consultant orthopaedic and reconstructive surgeon. I trained in medicine at [Ibadan], completed my orthopaedic training in the United Kingdom, and hold the FRCS (Tr. & Orth.). I went on to sub-specialist fellowships in South Africa and the north of England, and a national clinical leadership fellowship in arthroplasty and trauma. I also hold a Master of Public Health from London.

Over more than twenty years, across three countries, I have concentrated on the hip and the knee, and in particular on the complex and revision work that many surgeons prefer to avoid. I now bring that experience home, at Osteon Clinics in Lagos and at partner hospitals in Lagos and Abuja.

How I practise. I believe patients deserve two things that are not always given: the truth, and time. I will tell you plainly whether an operation will help you, and I will not recommend one you do not need. When surgery is right, I will do it to the standard I was trained to, and I will see it through your recovery, not just to the theatre door.

(Keep the "3 countries, 20 years" and the institutions visible. This is the trust anchor.)

4.5 For Referring Doctors

Headline: For referring doctors

Intro: If you have a patient with a joint that needs replacing, a revision, an infected implant, a difficult fracture or a case that has not gone to plan elsewhere, I would be glad to see them. Complex and revision work is the core of my practice.

What I offer your patient, and you

- A prompt, considered opinion, and an honest one. If your patient does not need surgery, I will tell you both.
- Specialist capability in revision, infection and complex reconstruction, to an international standard, without your patient leaving Nigeria.
- **Your patient comes back to you.** I treat the problem I am asked to treat and return your patient to your care, with a clear letter, every time. I am not looking to take over their care.

More than referrals: a clinical community



Join a community of doctors built around the hard cases. Log in to refer patients and follow their progress, bring your difficult cases to our monthly complex-case MDT, learn technique and management from the case library and live sessions, and, for those who want it, train toward a fellowship in arthroplasty. It is free to join, and open to verified doctors.

Buttons: Refer a patient • Doctor login / Join the community

How to refer

[Refer in under a minute through the portal, or by direct WhatsApp, phone or email. Same-week response, and a letter back every time.]

(This page is the shop window for the Clinical Community Portal, quietly the most commercially important asset on the site. See the referrer-engine playbook and the portal specification.)

4.6 For Patients Abroad / Diaspora

Headline: Care for your family in Nigeria, and for you when you visit

Intro: Many of my patients are cared for on behalf of family abroad, or are themselves visiting Nigeria and want treatment while they are here. For the operations people once travelled overseas for, revision, complex reconstruction, and joint replacement, that expertise is now available at home.

For families abroad

If a parent or relative in Nigeria needs a joint replaced or a complex problem addressed, I can assess them, keep you fully informed at each step, and manage their care and recovery. You do not need to be in the country for them to be in good hands.

Coordinating a visit [for diaspora patients timing surgery around a trip: assessment ahead of arrival, scheduling, recovery window, follow-up on return]

Button: Arrange care from abroad

4.7 Plan Your Care (conversion hub)

Headline: Plan your care

Intro: Here is exactly what happens, so there are no surprises.



1. **Get in touch.** Book a consultation online, by WhatsApp or by phone.
2. **Be assessed properly.** A full consultation and any imaging needed, then a clear, honest recommendation, including if the answer is not surgery.
3. **Understand the plan and the cost.** You will know what is advised, what it involves, and what it costs, in writing, before you commit. [Cost guidance / financing options to confirm.]
4. **Treatment.** At Osteon Clinics or a partner hospital, to an international standard.
5. **Recovery, with support.** Rehabilitation and follow-up, and a surgeon who stays reachable.

Button: Book a consultation **Line:** Or message us on WhatsApp

(If financing or fixed-price packages are offered, they surface here. The research plan will inform this.)

5. Proof and trust (build these in, do not bolt on)

- **Real outcomes**, even if simple to begin with: numbers of replacements and revisions, years, complication or revision rates if he tracks them. Confirmed and kept honest. This is the single most persuasive element and the direct answer to "4,000+ procedures."
- **Real patient stories**, with consent: the person, the problem, the result. One strong story beats ten adjectives.
- **Credentials, always visible:** FRCS, BOA fellowship, MPH, the institutions, the three countries.
- **Named affiliations:** Osteon, Cedarcrest, Diamed, shown as where care happens.
- **Google Business profile** claimed, complete, and gathering reviews. Most patients will check.

6. Search and discovery (SEO essentials)

Target the phrases patients and their families actually type. Own the ground the competitor is weak on: the complex and revision terms.

- **Priority terms:** "revision knee replacement Lagos", "revision hip replacement Nigeria", "infected joint replacement Nigeria", "failed knee replacement Nigeria", "second opinion joint replacement Lagos", "hip replacement Lagos", "knee replacement Lagos cost", "orthopaedic surgeon Lagos hip knee".
- Each service page targets a cluster of these in real, plain content, not keyword stuffing.
- **Insights pieces** (3 to 5 to start) built around real patient questions: "Do I really need a knee replacement?", "What to do when a joint replacement fails", "Is it safe to have your joint replacement in Nigeria?", "Signs your hip replacement may be infected". These earn search traffic and carry the honest-counsel voice.
- Fast load, clean mobile, proper page titles and descriptions, local business schema. The basics, done properly, will out-rank most of the field.

7. Build notes for designer and developer

- Mobile-first, fast on Nigerian mobile data, tested on mid-tier Android.
- WhatsApp as a first-class contact channel, not an afterthought.
- A working referral form and a working booking/enquiry form, going somewhere that is actually monitored, with a same-week response.
- Analytics from day one, so we can see what converts.



- Photography: a proper shoot of Dr Bola and of Osteon. This alone lifts the brand above most competitors.
- Keep the two-brand rule visible in every layout: **Dr Bola Akinola** is the masthead; **Osteon Clinics** appears as the place of care, endorsed, never dominant.

8. What we need to finalise the copy

- Confirmed outcome numbers (replacements, revisions, years, any rates he tracks).
- Two or three patient stories with consent.
- Cost guidance and any financing he is willing to offer.
- Medical school and exact training details to confirm the About page.
- A photography session.

These come out of the discovery questionnaire and a short content call. Copy above is ready to fill and refine the moment those land.